Narration Script

Right Hospital, Right Bed, Right Time · rationale video · version without narration

Length 2:01 · 1920 × 1080 · 30 fps · subtitles burned in · 261 words in four scenes. Read each line over **Right_Hospital_Right_Bed_Right_Time_Video_No_Narration.mp4**. Each line starts when its subtitle appears, and every visual cue is tied to a word, so match your pace to the subtitles.

How to record	Say it this way
1. Play the video and start speaking when each subtitle appears (**Start**). 2. Finish the line by **Finish by**. **Read time** is the length of the reference read in the narrated version. 3. Record each scene as its own take. Scene windows: 0:00–0:19, 0:19–0:40.5, 0:40.5–1:15.9, 1:15.9–2:01. 4. The boom lands at 1:43.3. Read the conclusion (4.7 to 4.10) after it, with a full pause before "Distance…", then stay silent to 2:01. 5. Pace is about 2.2 words per second, with a short breath at each comma.	**Albay**: al-BYE **jeepney**: JEEP-nee **Goals 3 and 11**: "Goals three and eleven" **e-mail**: EE-mail **"test network"**, never "Albay", for the A1 to A6, S3, S4 and S7 numbers **RA 10932**, ₱ fares and table values are shown on screen only. Do not read them.

Scene 1 · Trip

0:00.0–0:19.0 · 41 words · bar: Convenience

#	Start	Finish by	Read time	Narration (read this) · on screen
1.1	0:00.3	0:03.3	2.9 s	**Start with one trip, from point A to point B.** On screen: Albay map draws in; pins A and B drop; a dotted path with a "?" joins them.
1.2	0:03.8	0:07.5	3.7 s	**Which jeepney do you ride, when do you arrive, and what do you pay?** On screen: Three tags pop in, one per question: Which jeepney? · When do you arrive? · What do you pay?
1.3	0:08.1	0:13.4	5.2 s	**In Albay, no machine-readable record of routes, stops, or headways can tell you.** On screen: The map dims while unrecorded vehicles drive between towns. On "no machine-readable record" the tags turn grey; "R = S = H = empty set" (routes · stops · headways); chips Jeepney · Tricycle · UV van and "No routes, stops, or headways on record".
1.4	0:13.9	0:15.6	1.7 s	**Our timetable graph can.** On screen: Synthetic test network S1 (seven stops, schematic, not Albay) labelled "Timetable graph G = (V, E)"; gold route A3 → S4; card "A3 to S4: 18.6 min"; the fare meter counts to ₱20.00 for 7 km on a traditional jeepney, then "₱16.00 with 20% discount" and the fare formula (₱14 for the first 4 km, ₱2 per extra kilometre, fractions rounded up). Hold to 0:19.

Scene 2 · Community

0:19.0–0:40.5 · 48 words · bar: Fairness

#	Start	Finish by	Read time	Narration (read this) · on screen
2.1	0:19.2	0:20.2	0.9 s	**Zoom out.** On screen: The camera pulls back from S1 to the 18 cities and municipalities of Albay.
2.2	0:20.7	0:27.2	6.4 s	**The same model counts the hospital beds a person can reach in thirty minutes, and who is left out.** On screen: Landmarks and hospitals appear around the town centres (illustrative placement); the formula A_a(30) = Σ c_h · 1[T(o_a, h) ≤ 30 min]; 30-minute reach areas draw; landmarks inside light up and those outside grey out; “{ a : A_a(30) = 0 }” on “who is left out”.
2.3	0:27.6	0:34.3	6.5 s	**Sustainable Development Goals 3 and 11 ask for care without hardship and transport for all.** On screen: Lower third: “SDG 3.8 · care without financial hardship · SDG 11.2 · transport for all”.
2.4	0:34.7	0:39.7	4.9 s	**No figure yet counts the free, affordable beds each community can reach.** On screen: A “?” appears over the island that no reach area covers.

Scene 3 · Patient

0:40.5–1:15.9 · 70 words · bar: A life

#	Start	Finish by	Read time	Narration (read this) · on screen
3.1	0:40.8	0:42.3	1.4 s	**Now, an emergency.** On screen: The map zooms back into S1; the patient appears at A3 with a red pulse.
3.2	0:42.9	0:49.0	6.0 s	**The nearest hospital may be full, lack the needed service, or cost more than the family can pay.** On screen: A red dashed line marks the nearest hospital, S3 (7.0 min), with h_near = argmin T(A3, h) = S3. On "full" S3 turns red; card S3 · FULL · Level 1 · 0 free beds · 7.0 min · cost class 1.
3.3	0:49.6	0:54.1	4.3 s	**Maps show where hospitals are, not free beds, services, or cost.** On screen: Card "A map shows where. It does not show free beds, services, or cost." S4 shows "free beds ? · services ? · cost ?".
3.4	0:54.9	1:02.5	7.5 s	**E-mail agents collect free-bed reports,** [0:57.5] **and the ambulance goes to the nearest suitable hospital, with backups.** On screen: "HOSPITAL MODE"; e-mails from S3, S4 and S7 bring the counter to 3/3; S4 turns gold: Level 3 · 8 free beds · 18.6 min · cost class 0 (Malasakit Center); the suitability rule Φ appears and the ambulance carries the patient past S3 to S4 (h* = S4); backup S7 · Level 2 · 9 free beds · 51.4 min; five checks: level, service, ER, free bed, cost or emergency.
3.5	1:03.3	1:06.8	3.4 s	**In an emergency, payment never screens the patient.** On screen: Chip "Payment never screens an emergency (RA 10932)" with κ in K_E = {1, 2}. After the line: "Distance alone sent 5 of 6 anchors to a full hospital".
3.6	1:08.0	1:13.9	5.8 s	**When 15 crash victims compete for 17 free beds, the model places all 15.** On screen: Surge on S1 (Table 30): 15 emergency patients, 17 free beds. Each to own nearest: S4 gets 12 for 8 beds and 4 are turned away. On "places all 15": optimal assignment S4: 8 · S7: 7, all 15 placed.

Scene 4 · Why it must be right

1:15.9–2:01.0 · 102 words · bar: Proof

#	Start	Finish by	Read time	Narration (read this) · on screen
4.1	1:16.0	1:19.0	2.9 s	**The higher the stakes, the more the answer must be right.** On screen: The stakes ladder climbs to Proof; the right-edge bar fills.
4.2	1:19.3	1:23.2	3.8 s	**Earlier informal-transport projects began with field mapping.** On screen: “Earlier informal-transport projects”, then on “field mapping”: Field survey → Model.
4.3	1:23.5	1:29.3	5.7 s	**We reversed the order:** [1:25.1] **what can be defined, proven, and computed from open data alone?** On screen: On “reversed” the arrow flips to Model → Field survey; DEFINE, PROVE, COMPUTE appear on each word.
4.4	1:29.6	1:34.2	4.6 s	**From open maps, registries, census counts, and fares, we built the model.** On screen: Strip draws on each data source: Open data → Timetable graph → Access and inequality → Hospital mode.
4.5	1:34.4	1:37.3	2.8 s	**Eighteen results, proven, then verified.** On screen: “18 results = 15 propositions + 3 corollaries”; eighteen result tiles (Props. 1–3, 5–7, 9, 10, 12–18; Cors. 4, 8, 11) get a proof mark on “proven” and a check on “then verified”.
4.6	1:37.7	1:43.2	5.4 s	**Hospital routing matched independent code in all 10,400 queries.** On screen: The counter rolls to “10,400 / 10,400 routing queries matched”; chips: 11,474 / 11,474 catchment vertices agree · 200 / 200 forged replies rejected · +202 patients vs first-come · greedy sites ≥ 82.5% of optimum (proof ≥ 63.2%). The tiles then implode into a glowing core as a riser builds.
4.7	1:43.8	1:46.3	2.5 s	**This model will not add one bed to Albay.** On screen: Conclusion. A boom at 1:43.3 clears the screen to a plain background. The line appears large, with “one bed” in gold. The large text is its subtitle.
4.8	1:46.7	1:52.0	5.3 s	**But once hospitals report, it will tell him which one has a free bed and the right care.** On screen: Appears under it in two rows, with “free bed” and “right care” in gold. Both lines then fade out.
4.9	1:53.1	1:55.6	2.4 s	[pause] **Distance tells you where the hospital is.** On screen: [pause] The screen is empty for a moment. Then the line appears, with the paper title below.
4.10	1:56.0	1:58.7	2.7 s	**Access tells you whether it can help in time.** On screen: Lands in large gold type with a final hit; “in time” is underlined. This is the last frame. Hold silence to 2:01.

Clean read

The same narration without the production notes. Use it as a teleprompter or to rehearse. A slash marks a short breath.

SCENE 1 · TRIP · 0:00.0–0:19.0

0:00.3 Start with one trip, / from point A to point B.

0:03.8 Which jeepney do you ride, / when do you arrive, / and what do you pay?

0:08.1 In Albay, / no machine-readable record of routes, / stops, / or headways can tell you.

0:13.9 Our timetable graph can.

SCENE 2 · COMMUNITY · 0:19.0–0:40.5

0:19.2 Zoom out.

0:20.7 The same model counts the hospital beds a person can reach in thirty minutes, / and who is left out.

0:27.6 Sustainable Development Goals 3 and 11 ask for care without hardship and transport for all.

0:34.7 No figure yet counts the free, / affordable beds each community can reach.

SCENE 3 · PATIENT · 0:40.5–1:15.9

0:40.8 Now, / an emergency.

0:42.9 The nearest hospital may be full, / lack the needed service, / or cost more than the family can pay.

0:49.6 Maps show where hospitals are, / not free beds, / services, / or cost.

0:54.9 E-mail agents collect free-bed reports, / and the ambulance goes to the nearest suitable hospital, / with backups.

1:03.3 In an emergency, / payment never screens the patient.

1:08.0 When 15 crash victims compete for 17 free beds, / the model places all 15.

SCENE 4 · WHY IT MUST BE RIGHT · 1:15.9–2:01.0

1:16.0 The higher the stakes, / the more the answer must be right.

1:19.3 Earlier informal-transport projects began with field mapping.

1:23.5 We reversed the order: / what can be defined, / proven, / and computed from open data alone?

1:29.6 From open maps, / registries, / census counts, / and fares, / we built the model.

1:34.4 Eighteen results, / proven, / then verified.

1:37.7 Hospital routing matched independent code in all 10,400 queries.

1:43.8 **This model will not add one bed to Albay.**

1:46.7 **But once hospitals report, it will tell him which one has a free bed and the right care.**

[pause]

1:53.1 **Distance tells you where the hospital is.**

1:56.0 **Access tells you whether it can help in time.**